



LABORATORY REPORT



Name : Mrs. SWETAL SHAH	Sex/Age : Female / 44 Years	Case ID : 50200100075
Ref. By : D. HETAN SHAH	Dis. At :	Pt. ID : 411858
Bill. Loc. :		Pt. Loc. :
Reg Date and Time : 01-Feb-2025 07:25	Sample Type :	Mobile No. : 9601756054
Sample Date and Time : 01-Feb-2025 07:25	Sample Coll. By : NSRL	Ref Id1 :
Report Date and Time :	Acc. Remarks : -	Ref Id2 :

Abnormal Result(s) Summary

Test Name	Result Value	Unit	Reference Range
Glyco Hemoglobin (HbA1c)			
HbA1C	5.90	% of total Hb	<5.7: Normal 5.7-6.4: Prediabetes >=6.5: Diabetes
Haemogram (CBC)			
Total WBC Count	12540	/μL	4000.00 - 10000.00
Neutrophil	8314	/μL	2000.00 - 7000.00
Lymphocyte	3285	/μL	1000.00 - 3000.00
Lipid Profile			
HDL Cholesterol	34.27	mg/dL	40 - 60
Chol/HDL	4.66		0 - 4.1
Liver Function Test			
S.G.O.T.	34.05	U/L	5.0 - 34.0
Gamma Glutamyl Transferase	42.75	U/L	0 - 38
Renal Function Test			
Urea	8.51	mg/dL	14.98 - 40.01
Urine Examination			
Leucocytes (ESTERASE)	++		Negative
Blood	++		Negative
Leucocyte	22	/HPF	Nil
RBC (Isomorphic)	5	/HPF	Nil

Abnormal Result(s) Summary End

Note:(LL-VeryLow,L-Low,H-High,HH-VeryHigh ,A-Abnormal)

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MC-6136

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Bill. Loc. :		Pt. Loc. :
Reg Date and Time : 01-Feb-2025 07:25	Sample Type : Whole Blood EDTA	Mobile No. : 9601756054
Sample Date and Time : 01-Feb-2025 07:25	Sample Coll. By : NSRL	Ref Id1 :
Report Date and Time : 01-Feb-2025 08:51	Acc. Remarks : -	Ref Id2 :

TEST	RESULTS	UNIT	BIOLOGICAL REF. INTERVAL	REMARKS
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HAEMOGRAM REPORT

HB AND INDICES

Haemoglobin	12.1	G%	12.00 - 15.00
RBC (Electrical Impedance)	4.17	millions/cumm	3.80 - 4.80
PCV(Calc)	36.90	%	36.00 - 46.00
MCV (RBC histogram)	88.5	fL	83.00 - 101.00
MCH (Calc)	29.1	pg	27.00 - 32.00
MCHC (Calc)	32.8	gm/dL	31.50 - 34.50
RDW (RBC histogram)	13.30	%	11.00 - 16.00

TOTAL AND DIFFERENTIAL WBC COUNT (Flowcytometry)

Total WBC Count	H 12540	/μL	4000.00 - 10000.00
Neutrophil	[%] 66.3	%	EXPECTED VALUES 40.00 - 70.00
Lymphocyte	26.2	%	20.00 - 40.00
Eosinophil	2.9	%	1.00 - 6.00
Monocytes	4.4	%	2.00 - 10.00
Basophil	0.2	%	0.00 - 2.00

Neut/Lympho Ratio (NLR)	2.53		0.78 - 3.53		

PLATELET COUNT (Optical)

Platelet Count	353000	/μL	150000.00 - 410000.00
MPV	9.30	fL	6.5 - 12
PDW	15.6	fL	15 - 17

SMEAR STUDY

RBC Morphology	Normocytic Normochromic RBCs.
WBC Morphology	Leucocytosis, Neutrophilia, Lymphocytosis

Note:(LL-VeryLow,L-Low,H-High,HH-VeryHigh ,A-Abnormal)

Dr. Niyur Nagori

M. D. Path G-6476

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Platelet Platelets are adequate in number.
Parasite Malarial Parasite not seen on smear.

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TEST	RESULTS	UNIT	BIOLOGICAL REF RANGE	REMARKS
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HAEMATOLOGY INVESTIGATIONS

ESR <i>Photometrical capillary stopped flow kinetic analysis</i>	19	mm after 1hr	3 - 20
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BIOCHEMICAL INVESTIGATIONS

Plasma Glucose - F <i>Photometric, Hexokinase</i>	85.40	mg/dL	70.0 - 100
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Glycated Haemoglobin Estimation

HbA1C <i>HPLC</i>	H 5.90	% of total Hb	<5.7: Normal 5.7-6.4: Prediabetes >=6.5: Diabetes
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Estimated Avg Glucose (3 Mths) <i>Calculated</i>	122.63	mg/dL	Not available
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Please Note change in reference range as per ADA 2021 guidelines.

Interpretation :

HbA1C level reflects the mean glucose concentration over previous 8-12 weeks and provides better indication of long term glycemic control.

Levels of HbA1C may be low as result of shortened RBC life span in case of hemolytic anemia.

Increased HbA1C values may be found in patients with polycythemia or post splenectomy patients.

Patients with Homozygous forms of rare variant Hb(CC,SS,EE,SC) HbA1c can not be quantitated as there is no HbA.

In such circumstances glycemic control can be monitored using plasma glucose levels or serum Fructosamine.

The A1c target should be individualized based on numerous factors, such as age, life expectancy, comorbid conditions, duration of diabetes, risk of hypoglycemia or adverse consequences from hypoglycemia, patient motivation and adherence.

Skeletal Profile

Calcium <i>OCPC</i>	8.70	mg/dL	8.4 - 10.2
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Phosphorus Inorganic <i>Phosphomolybdate</i>	3.10	mg/dL	2.3 - 4.7
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Note:(LL-VeryLow,L-Low,H-High,HH-VeryHigh ,A-Abnormal)

Harveen

Dr Harveen Bhusari

MD Pathology (G-62229)

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TEST	RESULTS	UNIT	BIOLOGICAL REF RANGE	REMARKS
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BIOCHEMICAL INVESTIGATIONS

Renal Function Test

Urea <i>Urease</i>	L 8.51	mg/dL	14.98 - 40.01
Creatinine <i>Jaffe - Kinetic</i>	0.68	mg/dL	0.5 - 1.1
Uric Acid <i>Uricase</i>	5.38	mg/dL	2.6 - 6.0
Sodium <i>ISE</i>	138.00	mmol/L	136 - 145
Potassium <i>ISE</i>	3.70	mmol/L	3.5 - 5.1
Chloride <i>ISE</i>	105.00	mmol/L	98 - 107

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TEST	RESULTS	UNIT	BIOLOGICAL REF RANGE	REMARKS
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BIOCHEMICAL INVESTIGATIONS

Liver Function Test

S.G.P.T. NADH (Without P-5-P)	28.93	U/L	< 34	
S.G.O.T. NADH (Without P-5-P)	H 34.05	U/L	5.0 - 34.0	
Alkaline Phosphatase Enzymatic, PNPP-AMP	68.85	U/L	46.0 - 122.0	
Gamma Glutamyl Transferase Multipoint Rate-L-y-Glytamyl-p-nitroanilide	H 42.75	U/L	0 - 38	
Proteins (Total) Biuret	7.45	gm/dL	6.40 - 8.30	
Albumin (BCG)	4.14	gm/dL	3.5 - 5.2	
Globulin	3.31	gm/dL	2 - 4.1	
A/G Ratio Calculated	1.25		1.0 - 2.1	
Bilirubin Total Colorimetric Diazo Method	0.34	mg/dL	0.3 - 1.2	
Bilirubin Conjugated Colorimetric Diazo Method	0.13	mg/dL	0 - 0.50	
Bilirubin Unconjugated	0.21	mg/dL	0 - 0.8	

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BIOCHEMICAL INVESTIGATIONS

Lipid Profile

Cholesterol <i>Enzymatic</i>	159.53	mg/dL	110 - 200
HDL Cholesterol <i>Accelerator Selective Detergent</i>	L 34.27	mg/dL	40 - 60
Triglyceride <i>Arsenazo - Colorimetric</i>	126.52	mg/dL	<150
VLDL	25.30	mg/dL	10 - 40
Chol/HDL <i>Calculated</i>	H 4.66		0 - 4.1
LDL Cholesterol <i>Enzymatic</i>	99.96	mg/dL	0.00 - 100.00

NEW ATP III GUIDELINES (MAY 2001), MODIFICATION OF NCEP

LDL CHOLESTEROL	CHOLESTEROL	HDL CHOLESTEROL	TRIGLYCERIDES
Optimal<100	Desirable<200	Low<40	Normal<150
Near Optimal 100-129	Border Line 200-239	High >60	Border High 150-199
Borderline 130-159	High >240	-	High 200-499
High 160-189	-	-	-

- LDL Cholesterol level is primary goal for treatment and varies with risk category and assesment
- For LDL Cholesterol level Please consider direct LDL value
Risk assesment from HDL and Triglyceride has been revised. Also LDL goals have changed.
- Detail test interpretation available from the lab
- All tests are done according to NCEP guidelines and with FDA approved kits.
- LDL Cholesterol level is primary goal for treatment and varies with risk category and assesment

Note:(LL-VeryLow,L-Low,H-High,HH-VeryHigh ,A-Abnormal)

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Dr Harveen Bhusari

MD Pathology (G-62229)

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Sample Date and Time : 01-Feb-2025 07:25	Sample Coll. By : NSRL	Ref Id1 :
Report Date and Time : 01-Feb-2025 10:07	Acc. Remarks : -	Ref Id2 :

TEST	RESULTS	UNIT	BIOLOGICAL REF RANGE	REMARKS
Triiodothyronine (T3) CMIA	1.09	ng/mL	0.64 - 1.52	
Thyroxine (T4) CMIA	10.65	µg/dL	4.87 - 11.72	
TSH CMIA	1.62	µIU/mL	0.35 - 4.94	

INTERPRETATIONS

- Circulating TSH measurement has been used for screening for euthyroidism, screening and diagnosis for hyperthyroidism & hypothyroidism. Suppressed TSH (<0.01 µIU/mL) suggests a diagnosis of hyperthyroidism and elevated concentration (>7 µIU/mL) suggest hypothyroidism. TSH levels may be affected by acute illness and several medications including dopamine and glucocorticoids. Decreased (low or undetectable) in Graves disease. Increased in TSH secreting pituitary adenoma (secondary hyperthyroidism), PRTN and in hypothalamic disease thyrotropin (tertiary hyperthyroidism). Elevated in hypothyroidism (along with decreased T4) except for pituitary & hypothalamic disease.
- Mild to modest elevations in patient with normal T3 & T4 levels indicates impaired thyroid hormone reserves & incipient hypothyroidism (subclinical hypothyroidism).
- Mild to modest decrease with normal T3 & T4 indicates subclinical hyperthyroidism.
- Degree of TSH suppression does not reflect the severity of hyperthyroidism, therefore, measurement of free thyroid hormone levels is required in patient with a suppressed TSH level.

CAUTIONS

Sick, hospitalized patients may have falsely low or transiently elevated thyroid stimulating hormone. Some patients who have been exposed to animal antigens, either in the environment or as part of treatment or imaging procedure, may have circulating antianimal antibodies present. These antibodies may interfere with the assay reagents to produce unreliable results.

TSH ref range in pregnancy Reference range (microIU/ml)

First trimester 0.24 - 2.00

Second trimester 0.43-2.2

Third trimester 0.8-2.5

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Dr. Amit Prajapati

DCP.(G-17008)

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Interpretation Note:

Ultra sensitive-thyroid-stimulating hormone (TSH) is a highly effective screening assay for thyroid disorders. In patients with an intact pituitary-thyroid axis, s-TSH provides a physiologic indicator of the functional level of thyroid hormone activity. Increased s-TSH indicates inadequate thyroid hormone, and suppressed s-TSH indicates excess thyroid hormone. Transient s-TSH abnormalities may be found in seriously ill, hospitalized patients, so this is not the ideal setting to assess thyroid function. However, even in these patients, s-TSH works better than total thyroxine (an alternative screening test). When the s-TSH result is abnormal, appropriate follow-up tests T4 & free T3 levels should be performed. If TSH is between 5.0 to 10.0 & free T4 & free T3 level are normal then it is considered as subclinical hypothyroidism which should be followed up after 4 weeks & If TSH is > 10 & free T4 & free T3 level are normal then it is considered as overt hypothyroidism.

Serum triiodothyronine (T3) levels often are depressed in sick and hospitalized patients, caused in part by the biochemical shift to the production of reverse T3. Therefore, T3 generally is not a reliable predictor of hypothyroidism. However, in a small subset of hyperthyroid patients, hyperthyroidism may be caused by overproduction of T3 (T3 toxicosis). To help diagnose and monitor this subgroup, T3 is measured on all specimens with suppressed s-TSH and normal FT4 concentrations.

Normal ranges of TSH & thyroid hormones vary according trimester in pregnancy.

TSH ref range in Pregnancy	Reference range (microIU/ml)
First trimester	0.24 - 2.00
Second trimester	0.43-2.2
Third trimester	0.8-2.5

	T3	T4	TSH
Normal Thyroid function	N	N	N
Primary Hyperthyroidism	↑	↑	↓
Secondary Hyperthyroidism	↑	↑	↑
Grave's Thyroiditis	↑	↑	↑
T3 Thyrotoxicosis	↑	N	N/↓
Primary Hypothyroidism	↓	↓	↑
Secondary Hypothyroidism	↓	↓	↓
Subclinical Hypothyroidism	N	N	↑
Patient on treatment	N	N/↑	↓

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Amit Prajapati

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DCP.(G-17008)

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TEST	RESULTS	UNIT	BRI	TEST	RESULTS	UNIT	BRI
URINE EXAMINATION (STRIP METHOD AND AUTOMATED IMAGE EVALUATION)							
<u>Physical and Chemical examination</u>							
Colour	Pale yellow			Glucose	Negative		Negative
Appearance	Clear			Ketone Bodies Urine	Negative		Negative
Sp.Gravity	1.011		1.003 - 1.035	Urobilinogen	Negative		Negative
pH	7.00		4.6 - 8	Bilirubin	Negative		Negative
Leucocytes (ESTERASE)	++		Negative	Nitrite	Negative		Negative
Protein	Negative		Negative	Blood	++		Negative
<u>Automated Microscopy</u>							
Leucocyte	22	/HPF	Nil	Granular Cast	Nil	/HPF	Nil
Leucocyte Clumps	Nil	/HPF		RBC cast	Nil	/HPF	Nil
RBC (Isomorphic)	5	/HPF	Nil	WBC Cast	Nil	/HPF	Nil
RBC (Dysmorphic)	Nil	/HPF	Nil	Fatty Cast	Nil	/HPF	Nil
<u>Epithelial cells</u>				Mixed Cast	Nil	/HPF	Nil
Squamous EC	Present +	/HPF	Present	<u>Crystals</u>			
NonSquamous EC	Nil	/HPF		Crystals	Nil	/HPF	Nil
Transitional EC	Nil	/HPF		Calcium oxalate Monohydrate	Nil	/HPF	Nil
Renal Tubular EC	Nil	/HPF		Calcium oxalate Dihydrate	Nil	/HPF	Nil
<u>Bacteria</u>				Triple phosphate	Nil	/HPF	Nil
Bacteria	Nil	/μL	Nil	Uric acid	Nil	/HPF	Nil
Bacteria Rods	Nil	/μL	Nil	Leucine	Nil	/HPF	Nil
Bacteria Cocci	Nil	/μL	Nil	Amorphous	Nil	/HPF	Nil
Yeast	Nil	/μL	Nil	Trichomonas	Nil	/μL	Nil
<u>Cast</u>							
Hyaline Cast	Nil	/HPF	Nil				
Pathological Cast	Nil	/HPF	Nil				
Hyaline granular cast	Nil	/HPF	Nil				

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----- End Of Report -----

For test performed on specimens received or collected from non-NSRL locations, it is presumed that the specimen belongs to the patient named or identified as labeled on the container/test request and such verification has been carried out at the point generation of the said specimen by the sender. NSRL will be responsible Only for the analytical part of test carried out. All other responsibility will be of referring Laboratory.

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